EasySLR Decision Brief

Oct 14, 2026 · @hrsh

We should spend the three discovery calls on Pharmacoevidence, Source Health Economics and ICON Health Economics, with Pharmacoevidence as the one-call pick.

1. Segment

We chose HEOR and SLR service providers: consultancies that run systematic literature reviews (SLRs) for pharma and health technology submissions.

- **Problem:** SLR screening and data extraction are slow. An AI platform could cut that time. (Assumption, to confirm with EasySLR.)
- **Users:** SLR project leads, HEOR managers and screening researchers.
- **Approvers:** practice or operations heads, plus group procurement or IT security in larger firms.
- **Alternative rejected:** pharma in-house HEOR teams. They offer more volume, but procurement is likely slower. This is our judgement; we did not research it.

2. Top three

1. Pharmacoevidence (Mohali, India). SLRs are core work, with 22 to 26 staff. Its own AI tool was used in a NICE UK submission (November 2025 poster). Why now: AI is already in their workflow, so review efficiency matters. Risk: they may not need an outside platform. First question: "Do you use any third-party SLR platform for projects your in-house tool does not cover?"

2. Source Health Economics (Oxford, UK). Health economics, SLR and HTA services, about 33 staff. Why now: it was sold to SCIRIS, so tool choices may be changing. Risk: decisions may now sit with SCIRIS, not Source. First question: "After the SCIRIS deal, who chooses your review software?"

3. ICON Health Economics (UK and US). The weakest of the three: two Verified facts, one from an expired job post. The tool says Research more, so we would gather hiring or volume evidence before booking a call. Risk: a large CRO's procurement and security review would likely slow a pilot. First question: "Is your SLR reviewer team still hiring, and how many reviews does it deliver a year?"

3. Deferred and excluded

- **Costello Medical: Defer.** Already built its own screening application.
- **Maven: Research more.** Legal entity and headcount unconfirmed; its site claims proprietary AI/ML tools.
- **Evalueserve: Exclude.** Sells LitVerse, a competing living-SLR platform.

4. Changed evidence

Source was sold to SCIRIS, a Waterland-backed group with over 300 staff. We had assumed a small independent firm. We kept Source in the top three but recorded that decisions may sit at group level, so the call should test that first.

5. One-call pick

Pharmacoevidence. The rule engine suggests Defer because of its in-house AI tool. We overrode that to a discovery call, with a written reason: to test whether they would use an external platform for review types their tool does not cover, or act as a channel for EasySLR.

This override is the weakest judgement in the brief. If they rule out third-party tools, switch the call to Source Health Economics. New information that would change our choice: evidence that they have no appetite for external tools, or that SCIRIS controls Source's tooling completely.

6. Outreach and discovery

Outreach draft (not to be sent):

> Hi team,
>
> I'm researching how SLR teams handle screening and extraction in health economics work, and your published work caught my attention. Would you be open to a 20-minute call next week about how your team handles review tooling? I'd come with specific questions, not a pitch.
>
> Best regards, Harsh Singh

Three discovery questions:

1. Which review tools do you use, and where does your in-house tool fall short?

2. Would you evaluate a third-party platform alongside your own tool, or only as a partner or channel?
3. Who decides on review software, and do group or client contracts limit the choice?

7. One-day pilot

Only if discovery confirms the need.

- **Morning:** set up a workspace for one live SLR and import its reference list (format to confirm with EasySLR).
- **Afternoon:** agree screening criteria, then screen about 100 records alongside the team's current method.

Success (placeholder, to agree with the team): at least 20% less screening time per record, and at least 90% agreement with the team's decisions.